4.2 Hypertensive Crisis

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I have a severe headache and blurred vision." **Objective** • BP 210/130 mmHg • Nausea • Confused	**ACTUAL** **Acute pain** related to severely elevated blood pressure as evidenced by severe headache, blurred vision and nausea.	BP brought down gradually as prescribed (usually no more than 25% in the first hour), with no neurological worsening.	• Put her in a quiet room with the head of the bed raised. • While IV therapy runs, check BP every 5–15 minutes. • Give the IV antihypertensive through an infusion pump as prescribed. • Check neurological status every hour. • Chest pain, more confusion or seizures must be reported.	BP down as prescribed, neurological status stable: **goal met**.
Subjective • "I am frightened." **Objective** • Restless • HR 112/min	**ACTUAL** **Anxiety** related to sudden severe symptoms as evidenced by fear and restlessness.	Within 1 hour she says she feels calmer and rests quietly.	• Explain the procedures and monitoring in simple words. • Stay with her through the acute phase. • Keep visitors few, noise low and lights dim. • Teach slow breathing.	Calmer: **goal met**.
Subjective • *No specific complaint* **Objective** • Very high • BP • Kidneys may be affected	**RISK** **Risk for decreased cardiac output** as evidenced by risk factors: increased afterload.	Urine output stays above 0.5 mL/kg/hour, and no chest pain.	• Follow urine output and renal function. • Watch the ECG. • Report chest pain or breathlessness.	Output adequate, no chest pain: **goal met**.